

SECTION: Certification**SUBJECT: Anthropometric Data**

Federal Regulations: §246.7 (e)(1), Guidelines for Growth Charts and Gestational Age Adjustment for Premature, Low Birth Weight and Very Low Birth Weight Infants

Nutrition Services Standard: 7

Resources: This on-line module discusses the importance of accuracy and reliability in taking anthropometric measurements <https://depts.washington.edu/growth/index.htm> (MCHB Growth Chart Training) and provides a review of appropriate anthropometric equipment selection, calibration.

POLICY

Anthropometric data shall be no more than 60 days old at the time of certification. If the participant/parent or guardian doesn't have current anthropometric measurements from his/her health care provider (HCP), ask the participant the date/month of the last or future visit to the HCP to determine if measurements obtained will fall within appropriate timeframes for CT-WIC data entry and to determine if the participant has a medical home.

Often for children ages 2 and older, if it is not time for a child's annual physical the insurance company will NOT cover a doctor's visit for height and weight check only.

WIC certification requirements must occur at no charge to the participant (certification without charge). Therefore, local agency staff must be equipped to weigh and measure participants and provide this service as long as there is evidence of ongoing health care.

**Anthropometric measurements
Infants and Children:**

- At certification, mid-certification and re-certification visits, the Competent Professional Authority (CPA) enters the anthropometric measurements of the infant or child participant in the Anthropometric Data grid on the CT-WIC Lab screen. Based on the data, CT-WIC auto-assigns the appropriate anthropometric risk factors. It is recommended that the CPA, verify the CT-WIC automated risk factor for those infants or children that require gestational age adjustment (GAA). At follow-up visits, anthropometric data can be updated on the CT-WIC Lab screen.
- For infants enrolled prior to age six (6) months who are certified until their first birthday, it is best practice to assess growth throughout the certification period, at the following ages:
 - Birth-4 weeks
 - 2-5 months
 - 6-8 months
 - 9-11 months
 - Infants with inadequate growth or rapid growth velocity may require more frequent measurements.

At a minimum, CT-WIC Anthropometrics grid should include birth weight and length, 6 months and 12-month measurements.

Growth of infants and children ages 0-24 months are assessed using the WHO Growth Standards, which are based on optimal growth, rather than a reference population as the 2000 Centers for Disease Control and Prevention (CDC) growth curves. Body Mass Index (BMI) for age or weight-for-stature for children 24 months to 5 years are assessed using the 2000 CDC 2-20 years gender specific growth charts. CT-WIC will display both the BMI and BMI-for-age percentile after data is entered. See below for samples of lab screen showing BMI values as well as 0-12 month minimum and best practice anthropometric data entries and growth charts respectively.

Infants/Children 0-23 months- minimum



Infants/Children 0-23 months- best practice

Anthropometric Data																	☒ English(SAE) ☐ Metric		
	* Anthro Date	* Cert Period	* By Whom	Act. Age	AGA	* lbs	* oz	?	* in	* 1/8	R/S	?	BMI	BMI/ Age	Wt/ Age	Ht/ Age	Wt/ Ln	? Reasons	Date Created
	08/16/2024	2/27/2024	WIC CPA	1 y, 6 m	1 y, 5 m	19	6	☐	29	4	R	☐	N/A	N/A	12.80	3.94	33.20		8/16/2024
	02/20/2024	2/27/2024	EM Record	1 y, 0 m	11 m, 11...	16	0	☐	27	0	R	☐	N/A	N/A	5.25	3.14	18.31		2/27/2024
	08/18/2023	3/21/2023	EM Record	6 m, 2 d	5 m, 9 d	13	9	☐	24	4	R	☐	N/A	N/A	13.61	14.44	31.30		8/18/2023
	06/20/2023	3/21/2023	EM Record	4 m, 4 d	3 m, 11 d	11	9	☐	23	2	R	☐	N/A	N/A	11.90	21.60	21.61		6/20/2023
	06/06/2023	3/21/2023	WIC CPA	3 m, 21 d	2 m, 28 d	11	6	☐	23	4	R	☐	N/A	N/A	17.92	51.47	9.71		6/6/2023
	03/16/2023	3/21/2023	EM Record	1 m, 0 d	1 w, 0 d	7	11	☐	20	0	R	☐	N/A	N/A	61.76	59.86	45.87		3/21/2023
	02/16/2023	3/21/2023		0 days	-21 days	5	9	☐	18	0		☐	N/A	N/A	0.00	0.00	0.00		3/21/2023



Children 2-5 years

Infant/Child Height/Weight										Growth Chart									
Anthropometric Data										☒ English(SAE) ☐ Metric									
* Anthro Date	* Cert Period	* By Whom	Act. Age	AGA	* lbs	* oz	?	* in	* 1/8	R/S	?	BMI	BMI/ Age	Wt/ Age	Ht/ Age	Wt/ Ln	? Reasons	Date Created	
10/28/2024	10/28/2024	WIC CPA	4 y, 5 m		42	3	☐	42	2	S	☐	16.62	80.86	78.42	65.59	N/A		10/28/2024	
08/30/2024	10/28/2024	EM Record	4 y, 3 m		40	6	☐	41	4	S	☐	16.48	77.54	73.26	58.60	N/A		10/28/2024	
05/24/2024	5/1/2024	WIC CPA	4 y, 0 m		40	3	☐	40	6	S	☐	17.02	86.53	80.42	57.91	N/A		5/24/2024	

Add Remove

* Birth Weight: lbs oz ☐ Unknown

* Completed Weeks of Gestation: ☐ Unknown

Immunization Status: ☐ Reviewed ☐ Referred

* Birth Length: in 1/8 ☐ Unknown

Weight Change: lbs oz

Height Change: in eighths

Time Interval: m, d

History of anthropometric assessments including BMI calculation and percentiles, shown as numerical values, can be viewed on the Lab Screen. CT-WIC shows a graphical representation of growth curves. This upgrade provides the CPA with an overview of the infant or child's growth over time.

To provide the parent, caretaker or guardian with a visual overview of the infant/child's growth, CPA's should display the growth chart and print a copy if necessary or requested by the parent, caretaker or guardian.

Additionally, for infants and children ages 0-24 months and children ages 2-5 year old it is important to clearly communicate with parents and caregivers what the specific growth curve conveys or represents, especially if the pediatrician or health care provider is using a different growth chart. Also, when a child transfers from the WHO Growth Standards to the 2000 CDC 2-20 years gender specific growth curves, some changes in percentiles may occur.

- Use of FNS Nutrition Risk Criterion #114 (Parent with BMI ≥ 30) is a manually assigned risk factor. If the mother's pre-pregnancy BMI is known, the CPA must manually assign this risk. CPA's are not required to request and/or calculate the BMI of each parent. However, if this risk is used, the parent's information should be documented in the Education Notes.

Note: It is possible for an infant or child to have CT-WIC FNS Nutrition Risk Criterion #103 (**Underweight or At Risk of Underweight**) and #114 (**Parent with BMI ≥ 30**) assigned simultaneously, based on the mother's assessed BMI and the infant or child's weight for length and BMI for age. Nutritionists are encouraged to use discretion and professional judgment when

discussing this with caregivers. Additional information can be found in the *CT WIC Program Motivational Interviewing Guidance Local Agency Resource*, and *Consistent Education Messages: Childhood Overweight and Obesity Guide for BMI Assessment and Effective Communication with Families*, guidance document.

Additional Resources:

Changes in Terminology for Childhood Overweight and Obesity:
<http://www.cdc.gov/nchs/data/nhsr/nhsr025.pdf>

WIC 200-13 Supplement to Infant/Children Certification Form, WIC 200-12 Infant and Children Nutrition Assessment Form Guidance for more information on counseling tips.

Also refer to CT Nutrition Risk Update presentation, related nutrition risk write-ups, and CDC’s WHO Growth Chart On-line training: [Training Module: Using WHO Child Growth Standards | Growth Chart Training | CDC](#) for more in-depth information.

Prenatal, Breastfeeding and Postpartum Participants:

At certification, the CPA records anthropometric measurements for manual and/or auto-assignment of anthropometric risk factors in the Lab screen in CT-WIC. To provide quality nutrition services, during follow-up appointments, enter current prenatal weight in Lab screen and click “Save”. CT-WIC will auto-calculate weeks’ gestation and incremental weight gain or loss and assign any applicable weight-based risks once new information is saved.
Verify measurements on the Prenatal Weight Gain Grid in CT-WIC at each prenatal visit. Discuss strategies to increase, decrease or maintain weight gain.

For Breastfeeding and Postpartum participants, current weight can also be updated at follow-up visit on the Lab Screen to determine if participant has returned to pre-pregnancy weight or reached their weight loss/gain goals.

Pregnant Participants

Anthropometric Data

☒ English(SAE) ☐ Metric

* Anthro Date	* Cert Period	* By Whom	* lbs	* oz	?	* in	* 1/8	?	Weeks	PG Wt Gain	Weight Gain/Loss	Cat	Pre-PG BMI	Current BMI	? Reasons	Date Created
01/17/2025	7/23/2024	WIC CPA	213	13	☐	64	0	☐	29	-0.19	0.81	PG	36.73	36.70		1/17/2025
11/11/2024	7/23/2024	MD/PA	213	0	☐	64	0	☐	21	-1	3	PG	36.73	36.56		11/21/2024
10/22/2024	7/23/2024	WIC CPA	210	0	☐	64	0	☐	18	-4	-6.81	PG	36.73	36.05		10/22/2024

AddRemove

* Month/Year of First Prenatal Visit:7 / 2024

* Pre-pregnancy Weight:214 lbs☐ Unknown

* Multifetal Gestation:☐ Yes ☒ No

* Previous Pregnancies?☒ Yes ☐ No

* Previous Live Births:1

* Last Pregnancy Ended:12/27/2021

* Miscarriages:☐ Yes ☒ No

* Stillbirth:☐ Yes ☒ No

* Other:☐ Yes ☒ No

Breast/Chest Feeding/Postpartum Participants

Anthropometric Data															English(SAE) ☒ Metric ☐	
* Anthro Date	* Cert Period	* By Whom	Weight			Height			Weeks	PG Wt Gain	Weight Gain/Loss	Cat	Pre-PG BMI	Current BMI	* Reasons	Date Created
10/14/2024	4/24/2024	WIC CPA	129	0	☐	63	0	☐			-1.38	BE	22.14	22.85		10/14/2024
06/12/2024	4/24/2024	WIC CPA	130	6	☐	63	0	☐			2.56	BE	22.14	23.09		6/12/2024
04/24/2024	4/24/2024	WIC CPA	127	13	☐	63	0	☐			-11.94	BE	22.14	22.64		4/24/2024

Add
Remove

* Month/Year of First Prenatal Visit: 8/ 2023

* Pre-pregnancy Weight: 125 lbs ☐ Unknown

* Weight at Delivery: 146 lbs

* Weight gained during pregnancy: 21 lbs

Actual Delivery Date: 3/18/2024

* Total Infants: 1

* Live at Birth: 1

* Pregnancy Outcome

☒ Live ☐ Miscarriage/Stillborn/Abortion

☐ Multiples ☐ Born alive but died within 28 days

Additional Resources:

WIC 200-13 Supplement to Women's Certification Form and 200-12 Nutrition Questionnaire and Assessment for Guidance for additional background and information on 2009 IOM Recommendations Prenatal Weight Gain and prenatal weight gain counseling tips.

Information is also available at the following link: [Weight Gain During Pregnancy: Reexamining the Guidelines | The National Academies Press](#)

<https://www.ncbi.nlm.nih.gov/pmc/articles/PMC3974574/table/T1/?report=objectonly>

Documentation:

If anthropometric measurements or blood work are obtained verbally from the HCP, documentation should be included in CT-WIC indicating the date the verbal order was obtained. Faxed information should be scanned into CT-WIC. *Please note that verbal anthropometric measurements or blood work are not acceptable from a WIC participant since this information is used to assign risks and eligibility.*

Quality Assurance:

During quarterly chart audits, program management should ensure that anthropometric data is being collected timely i.e., at regular intervals, and documented correctly in CT-WIC. This includes ensuring that the fields in the Lab Screen grid are populated correctly and the growth charts and/or weight gain grids display correctly.